Name	: Saroj Agarwal	Patient Id.	: 10663186
Date of Birth	: Aug 27, 1962	Age	: 63 years Sex : Female
Case Type	: Inpatient Case	Case Number	: 1003707451
Attending Physician	: Dr. Darshit Shah		

Inpatient Stay

Admission Date	**:** Feb 20, 2026	**Admission Time**	**:** 3:36:29 AM
Discharge Date	**:** Apr 8, 2026	**Discharge Time**	**:** 1:38:24 PM

Diagnosis

Diagnosis Description	Diagnosis Code
1.Ca Uterus	C54

Presenting Complaints & Brief History

CHIEF COMPLAINTS:-
Transferred from outside hospital for further management.
C/O - Breathlessness and coughing with bilateralpedal edema.
CXR - Left pleural effusion
ICD in situ since 12/2/26
Fluid - Atypical cells ?Large cell morphology

H/O Ca Uterus - Operated 3 years back

Major Laboratory Findings

labs noted

Procedure(s)

Echocardiography Portable
WB FDG PET CECT scan
Rehab med-chest therapy-ward
Rehab med-chest therapy-ward
USG Portable Kidney/Transplant Kidney/KUB
X Ray Portable Chest AP

Course in the Hospital

20.02.2026: Patient was reviewed on rounds with complaints of breathlessness and cough. Known case of carcinoma uterus with left pleural effusion and lung collapse with ICD in situ since 12/02/2026. 2D ECHO showed LVEF of 60% with severe PAH (PASP 78 mmHg). Whole body FDG PET scan and chest X-ray were advised and ICD drain was flushed. Weekly chemotherapy with Paclitaxel and Carboplatin (Cycle 1 Day 1) was initiated on 20.02.2026. Patient also had complaints of chest pain around ICD site; CXR was done and nebulisation, Inj. Cort 100 mg stat and Inj. Kabimol 1 g stat were administered. Reference to pulmonology and pigtail catheter assessment was given with advice for monitoring of ICD output and supportive care. Urine RM/CS and USG KUB were advised in view of mild hematuria.

21/02/2026 Patient was reviewed on rounds Labs were noted and Inj. Magnex Forte 3 g IV BD was continued. Urine culture was advised to be sent. Patient had complaints of cough with expectoration and on auscultation bilateral wheeze was present. Sputum culture was advised, Mucomix nebulisation was stopped and Tab Mucinac 600 mg TDS was initiated. Nebulisation with Duolin and Budecort was continued along with Syp Lupituss. Fluid restriction up to 1.5 L/day and 24-hour ICD drain output charting were advised. Urine was noted to be clear and antibiotics were continued with advice to review if hematuria persisted.

22.02.2026:
The patient developed acute difficulty in breathing with bilateral wheeze (left > right). Oxygen saturation was maintained on nasal prongs. She was managed conservatively with bronchodilator support and close respiratory monitoring.

23.02.2026:
She continued to have persistent cough and wheezing. The intercostal drain (ICD) was found blocked at the connection site and was flushed, relieving the obstruction. PET scan findings were reviewed. Sputum culture showed budding yeast with pseudohyphae, raising suspicion of fungal infection. Ultrasound thorax was advised for ICD localization and further decision-making. Infective etiology was addressed, and close monitoring was continued.

24.02.2026:
Stridor was suspected, and ENT evaluation was sought. Flexible nasopharyngolaryngoscopy revealed bilateral vocal cords in paramedian position with restricted mobility and minimal glottic chink. Close airway observation was advised with consideration for tracheostomy if required. The ICD was removed, and repeat chest X-ray was planned. Oxygen saturation was around 92% on 5 L oxygen, considered acceptable as per pulmonary assessment.

25.02.2026:

The patient had an episode of desaturation requiring increased oxygen support, and a MET call was raised. Irregular pulse was noted. Repeat ENT evaluation confirmed restricted vocal cord mobility (left more than right). A multidisciplinary discussion was conducted with the family regarding airway management options, including tracheostomy versus vocal cord lateralization. A decision was made to proceed with vocal cord lateralization after obtaining necessary clearances.

26.02.2026:
Preoperative pulmonary and cardiology assessments were completed. 2D echocardiography showed severe pulmonary hypertension (PASP ~72 mmHg) with preserved left ventricular ejection fraction (60%) and mild right ventricular dilatation. The patient underwent left vocal cord lateralization under general anesthesia. Intraoperatively, she developed desaturation requiring intubation. Post-procedure, she was shifted to ICU on mechanical ventilation. Postoperative ABG indicated hypoxemia. The pulmonary hypertension was attributed to underlying lung pathology (Group III). A plan for ventilatory support, cautious fluid management, and gradual weaning was initiated.

27.02.2026:
On postoperative day 1, the patient remained intubated and sedated in ICU, maintaining SpO_2 ~95% on FiO_2 60%. Bilateral lower zone crepitations were present, with decreased air entry at the left base. High oxygen requirement persisted. The plan included ventilator weaning, point-of-care ultrasound assessment, and screening echocardiography. ENT clearance for extubation was obtained, with final feasibility to be confirmed by the pulmonary team. Extubation to HFNC was planned once clinically appropriate. Hemodynamics were stable, and diuretic dosing was to be titrated based on urine output and blood pressure.

28.02.2026
The patient, a known case of carcinoma uterus with pulmonary hypertension and vocal cord lateralisation, remained admitted in ICU under a prioritization model. She was conscious and oriented. Vitals showed BP 99/61 mmHg, HR 75/min, and SpO_2 93% on HFNC (FiO_2 60%). Respiratory examination revealed bilateral air entry; abdomen was soft. Urine output was adequate (70–80 ml/hr). She continued to have high FiO_2 requirements. The management plan included fluid restriction (TFI 1.5 L with negative balance target of 1 L), diuretics, tapering of HFNC, early mobilization (EOB), continuation of Ryle’s tube feeds (40 ml/hr), and discontinuation of IV fluids and antiemetics.
The patient was conscious, oriented, and comfortable on HFNC with SpO_2 around 92% on FiO_2 55%, with gradual attempts to taper oxygen support. Local wound site was clean with intact dressing and no swelling. The plan was to continue the same management and RT feeds. A FEES study was planned once oxygen requirement decreases.
Steroid dose was reduced to dexamethasone 4 mg IV once daily after discussion with the treating consultant. Ongoing management was continued as per plan.

01.03.2026 (ICU Day 3 | POD 3 – Vocal Cord Lateralisation)
Patient was conscious, oriented, and hemodynamically stable on HFNC with moderate FiO_2 requirement. Chest examination revealed bilateral basal decreased air entry with left-sided

crepitations; imaging showed left mid-zone and bilateral lower-zone consolidation with left pleural effusion. Echocardiography demonstrated LVEF 60%, mildly dilated RV with normal function, mild TR, and pulmonary hypertension (PASP ~46 mmHg). Telemetry showed sinus rhythm with intermittent ventricular bigeminy. Oxygen support and diuresis were continued with a target negative fluid balance. Electrolytes were monitored closely. Chest physiotherapy and gradual mobilization were advised. Plan included tapering FiO_2 as tolerated and considering chemotherapy based on clinical stability.

02.03.2026
Patient remained conscious and afebrile with gradual improvement in oxygenation, though still requiring HFNC. Pleural effusion persisted but bedside ultrasound showed no significant drainable collection. Fluid balance was maintained negative with ongoing diuresis. Electrolytes and renal parameters remained stable. Mobilization to chair was initiated. Chemotherapy (Paclitaxel + Carboplatin + Dostarlimab, C1D8) was administered and tolerated well. Plan included continued respiratory support with gradual FiO_2 taper, physiotherapy, monitoring for desaturation, and reassessment for step-down once stable.

03.03.2026
Clinical status improved with reduction in oxygen requirement; transitioned toward nasal prongs with acceptable saturations. Breathing pattern and phonation showed improvement post vocal cord intervention. Repeat echocardiographic screening continued to show stable cardiac function with pulmonary hypertension. Left-sided pleural effusion and consolidation persisted but without clinical worsening. Negative fluid balance was maintained. FEES study was planned to assess swallowing. Patient was mobilized with oxygen support. Considered for transfer to wards based on continued stability.

04.03.2026
Patient on low-flow oxygen via nasal prongs, maintaining SpO_2 ~95% while sitting. Hemodynamically stable (BP 140/70 mmHg, HR 69/min). Respiratory examination revealed bilateral basal crepitations; left pleural effusion persisted. Oxygen requirement minimal with plan for further tapering. Diuretic dosing under reassessment based on fluid balance and potassium levels. FEES planned bedside. Antibiotic duration under review. Referred to Dr. Talha Meeran for ongoing pulmonary hypertension management. Handover completed with plan for continued monitoring, respiratory support optimization, mobilization, and possible transfer to ward once stable.

05.03.2026
Patient found to have pulmonary hypertension with vocal cord palsy; vocal cord lateralisation performed. Patient maintained SpO_2 ~95% on low-flow oxygen and tolerated pureed diet with RT feeds. Managed with diuretics, antibiotics, antifungals, and pulmonary HTN therapy (Tadalafil). Speech and swallow therapy initiated.

06.03.2026
Patient remained hemodynamically stable with persistent expiratory wheeze. Continued RT feeds,

chest physiotherapy, spirometry, and speech therapy. Cardiology review noted frequent PVCs and NSVT, and anti-arrhythmic therapy (Cordarone, Concor) was initiated with close monitoring.

07.03.2026
Clinical status improved, though wheeze and desaturation during feeds were noted; hence oral feeds were withheld and RT feeds continued. Managed conservatively with nebulisation, antibiotics, and chest physiotherapy. Arrhythmias improved with treatment.

08.03.2026
Patient developed episodes of polymorphic PVCs and VTach with palpitations. Managed with Xylocard infusion and anti-arrhythmic medications. Coronary angiography was considered but later deferred after cardiology discussion.

09.03.2026
Patient developed acute bronchospasm with laboured breathing, likely related to microaspiration. Managed with nebulisation, steroids, and supportive care, following which symptoms improved. Continued monitoring for arrhythmias and respiratory status.

10.03.2026
Patient clinically improved with systemic steroids. Telemetry showed sinus rhythm without significant arrhythmias. Continued anti-arrhythmic medications, nebulisation, and pulmonary HTN therapy.

11.03.2026
Patient remained hemodynamically stable, no bronchospasm in last 24 hours, and no ventricular arrhythmias in last 48 hours. SpO_2 100% on minimal oxygen support, with reduced bilateral wheeze on auscultation. Plan to wean oxygen and shift patient to the ward from ICU.

12.03.2026:
The patient, a known case of carcinoma uterus with pulmonary hypertension, status post vocal cord lateralisation, was under ICU care. She remained conscious, oriented, and hemodynamically stable, maintaining oxygen saturation around 90–91% on room air. Intermittent ventricular ectopy/bigeminy was noted on telemetry. Electrolyte correction with IV potassium and magnesium was administered. Anti-arrhythmic therapy including Concor, Cordarone, Mexiletine, Lasix, Aldactone, and Tadalafil was continued. Respiratory management included nebulisation, chest physiotherapy, incentive spirometry, and deep breathing exercises. Speech therapy evaluation was advised and oral feeding was supervised. The patient remained clinically stable with a plan for gradual steroid taper and continued ICU monitoring.

13.03.2026:
The patient continued to remain clinically stable with no respiratory distress and oxygen saturation maintained on room air. Telemetry showed sinus rhythm with intermittent ventricular bigeminy. Electrolyte correction was continued with a target K >4 and Mg^{2} >2. Respiratory exercises and

mobilisation were encouraged. The ICU team planned shift out of ICU with 72-hour Holter monitoring while continuing anti-arrhythmic and supportive therapy. Swallow therapy was continued.

14.03.2026:
The patient was shifted out of ICU to the ward in stable condition. She remained conscious, oriented, and hemodynamically stable, maintaining SpO_2 around 88–91% on room air, which was considered acceptable in the absence of distress. Occasional wheeze and radiological left mid-zone atelectasis were noted. Management included oral Wysolone, nebulisation therapy, respiratory physiotherapy, and mobilisation. Cardiology review recommended continuation of anti-arrhythmic medications and 72-hour Holter monitoring. Strict monitoring of vitals and intake–output charting was advised. Speech and swallow therapy was continued with gradual reintroduction of oral feeds. The patient was considered high risk for ICU readmission and the family was counselled accordingly.

15.03.2026:
The patient remained clinically stable and continued physiotherapy and supportive care. Review by the treating team was planned. Paclitaxel and Carboplatin chemotherapy was planned for the following day.

16.03.2026
The patient was reviewed and found to be clinically stable with sinus rhythm on ECG. Vitals were within acceptable limits. Mild trace pedal edema was noted without JVD. Serum potassium was low (3.78 mmol/L), and correction was initiated. A 72-hour smart cardio Holter monitoring was advised along with repeat ECG and early morning potassium monitoring.
On the same day, the oncology team reviewed the case, and the patient received the planned cycle of chemotherapy as per protocol with appropriate premedication and antiemetic support. Post-chemotherapy supportive care including growth factor support was advised.
Additionally, the patient reported improvement in cough and voice following expectoration. Speech and swallow therapy was advised to be intensified, with further review planned.

Tab APRECAP 125 mg 1 tab PO stat
Inj Dexa 8mg + Inj Pan 40mg + NS 100mlIv over 30mins
Inj Avil 2ml + Inj Hydrocort 100mg + NS 100ml IV over 30mins
Inj Paclitaxel (Mitotax) 100 mg + 5% Dextrose 250ml IV over 90 mins
Inj Carboplatin 180 mg + 5% Dextrose 500ml IV over 2 Hours
100 ml NS Flush

Day 2 - Inj Neukine 300mcg SC One dose (24 hours after chemotherapy)

Day 2-4
Tab Pan 40mg PO 1-0-1 (30min before food)
Tab Emeset 8mg PO 1-1-1
Cap Aprecap 80 mg Day 2 Day 3

17.03.2026
The patient was reassessed and remained hemodynamically stable with sinus rhythm on ECG (QTc 435 ms). Vitals were stable with adequate oxygen saturation on room air. Clinical examination revealed bilateral expiratory wheeze and trace pedal edema, with no evidence of JVD. Input-output charting suggested a negative fluid balance. Serum potassium showed improvement (4.18 mmol/L). Continuation of current management was advised, along with follow-up of the smart cardio monitoring report.

18.03.2026
The patient was advised to continue the ongoing treatment, with serum sodium and potassium planned for the following morning. Inj. Optineuron was administered in 100 ml normal saline over 4 hours. The patient was evaluated by the speech and swallow therapist, who noted a breathy but improving voice, along with improving breathing-swallow coordination. Adduction exercises were reviewed and continued, and a FEES study was planned. Subsequently, FEES was performed in the presence of the therapist, which demonstrated improved vocal cord approximation and voice quality, no overt aspiration, and minimal residual feed in the valleculae. The patient was advised to continue speech and swallow therapy.

19.03.2026
The patient was reviewed by Dr. Rajas Patel (locum for Dr. Darshit Shah) with complaints of mild throat irritation. Vitals were stable (BP 112/76 mmHg, PR 78/min, SpO_2 98% on room air), and continuation of the same treatment was advised. Later, the patient remained clinically stable with improvement in voice, and pleasure feeds were initiated. Mobilisation was encouraged. The patient requested a soft silicone nasogastric tube, which was planned to be inserted during the next endoscopic evaluation.

20.03.2026
A MET call was activated for chest pain; however, the patient remained hemodynamically stable and was managed symptomatically with nebulisation and analgesics, with no requirement for escalation of care. The patient remained in the ward, and the family was informed. Subsequently, the patient developed respiratory symptoms with bilateral expiratory wheeze. Vitals showed HR 84/min, BP 100/60 mmHg, and SpO_2 92% on room air, with input/output of 1200/1350 ml. Serum potassium was 4.79 mmol/L. A 12-lead ECG was advised. Treatment included Inj. Hydrocortisone 100 mg IV stat, nebulisation with Duolin (TDS) and Budecort (BD), and continuation of cardiac medications (Concor, Cordarone, Tadalafil, Mexiletine, Lasix, and Aldactone). Later, the patient was clinically stable; the case was discussed with the patient’s son. A plan was made to repeat FEES after the next cycle of immunotherapy and chemotherapy, and the patient was advised to continue pleasure feeds.

Supportive care with IV fluids and multivitamins administered.

21.03.2026 - 22.03.2026- Patient developed mild throat irritation followed later by desaturation requiring oxygen and IV steroids. Patient was planned for chemotherapy.

23.03.2026, she presented with acute breathlessness, cough, and hypoxia (SpO_2 77–78% on room air), improving with oxygen; clinical evaluation raised suspicion of pulmonary embolism or cardiac etiology, and investigations including CTPA, 2D Echo, ABG, and NT-proBNP were advised. FEES study revealed reduced laryngeal sensation, and she was placed on puree diet with Ryle's tube feeding and speech/swallow therapy.

24.03.2026- anticoagulation with Inj. Clexane was initiated and CTPA was performed.

25.03.2026, imaging (CTPA and 2D Echo) confirmed a saddle-shaped submassive pulmonary embolism with elevated NT-proBNP and oxygen dependency; Troponin I was advised to assess need for ICU-based thrombolysis. The patient continues on anticoagulation, oxygen support, and nebulization, with close monitoring. A prior MET call on 25.02.2026 for desaturation was managed successfully with oxygen and nebulization, and the patient remained stable thereafter.

26.03.2026
Hemodynamics stabilized gradually; urine output remained adequate.
Managed with ventilatory support, antibiotics, and close monitoring.

27.03.2026
Patient remained intubated initially, with persistent circulatory shock requiring vasopressors.
Noted ET tube bleeding and drop in hemoglobin; anticoagulation continued to be withheld.
Gradual improvement in oxygenation and hemodynamics observed.
Vasopressors were tapered, and sedation reduced with plan for weaning.
Continued supportive care including transfusion, electrolyte management, and enteral feeding.

28.03.2026
Successfully extubated after clinical improvement.
Post-extubation period remained stable on low-flow oxygen support.
Vasopressors continued at reduced doses.
Hemoglobin drop persisted; transfusion support provided.
Central venous line placed for ongoing management.
Minimal residual airway bleeding noted, showing improvement.

29.03.2026
Patient conscious and maintaining oxygenation on nasal prongs.
Mild hemoptysis persisted, with associated drop in hemoglobin.
Continued need for vasopressor support, though at lower doses.
Anticoagulation (clexane) remained on hold due to bleeding risk and thrombocytopenia.
Supportive care continued with transfusions, chest physiotherapy, and monitoring.

30.03.2026
Hemodynamically more stable; vasopressin discontinued, minimal noradrenaline support ongoing.
Oxygen requirement persisted (4–5 L/min).
Mild hemoptysis continued.
Platelet counts showed improvement; consideration to restart anticoagulation.
Gradual mobilization and pulmonary rehabilitation initiated.

31.03.2026
Patient remained in ICU with persistent but improving circulatory shock, on low-dose noradrenaline.
Developed new right middle and lower lobe consolidation on chest imaging, suggestive of infection.
One episode of fever noted; sepsis workup initiated.
Broad-spectrum antibiotics (Zavicefta and Aztreonam) and steroids started.
Anticoagulation (Clexane) restarted cautiously as platelets improved.
Continued oxygen support, chest physiotherapy, and gradual mobilization.
Overall condition guarded with ongoing need for close monitoring.

01.04.2026
Oxygen requirement gradually reduced (6L → 2L). Mild hemoptysis noted. Patient conscious, oriented, and hemodynamically improving. Thrombocytopenia showed improvement. Management included anticoagulation planning, fluid balance, chest physiotherapy, and nutritional optimization.

02.04.2026
Clinical improvement continued. Noradrenaline tapered and eventually stopped by evening. Oxygen requirement maintained at 2L with stable saturation (>93%). Hemoptysis resolved. Patient mobilized, urine output adequate, and vitals stable. Plan included continued anticoagulation, chest physiotherapy, and monitoring for hemodynamic stability.

03.04.2026
Patient remained stable without inotropic support for >24 hours. Oxygen requirement reduced to 1–2L. No further hemoptysis. Chest X-ray showed significant improvement. Electrolyte imbalance (hypokalemia) required correction. Therapeutic anticoagulation (Clexane) planned/escalated. Considered fit for transfer from ICU to ward.

04.04.2026
Patient hemodynamically stable with no vasopressor requirement. Maintaining SpO_2 ~92–93% on 2L oxygen. Post pulmonary thrombectomy status with high PESI risk but clinically improving. Electrolyte correction ongoing. Anticoagulation continued (Clexane). Advised chest physiotherapy and mobilization. Deemed stable for transfer to ward.

05.04.2026
The patient was evaluated and found to be clinically stable with no hemoptysis. Oral liquids were

initiated, and the patient tolerated intake without cough or respiratory distress. Oxygen support was required at 2 L/min, though the requirement had decreased. Vitals included HR 81/min, BP 118/60 mmHg (on noradrenaline support), and SpO_2 99%. Intake/output was balanced. Laboratory results showed sodium 151.20, potassium 3.40, hemoglobin 9.8, total leukocyte count 3870, and platelets 93,000. Advice included continuation of current management, administration of 400 ml water via Ryle's tube for hypernatremia, gradual tapering of oxygen (0.5 L/min every 6–8 hours to maintain SpO_2 >95%), and repeat CBC and chest X-ray the following morning.

06.04.2026
The patient remained stable with no new complaints. Respiratory status improved significantly, with decreased distress and a clear respiratory system on examination. The patient was able to eat without coughing. Chest X-ray showed improvement. Oxygen support was being tapered while maintaining SpO_2 above 95%, and mobilization around the bed was advised. Discharge was planned for the following morning from a pulmonary standpoint.
On the same day, chemotherapy (Cycle 3, Day 1) was initiated as per protocol, including Paclitaxel and Carboplatin with appropriate premedication (Aprecap, dexamethasone, pantoprazole, avil, hydrocortisone) and IV fluids. Post-chemotherapy care included a normal saline flush and planned administration of Neukine 300 mcg subcutaneously 24 hours later. Additional instructions included conducting CBC, sodium, and potassium tests, increasing Clexane to 0.6 ml subcutaneously twice daily, and continuing Zavicefta and azithromycin. Discharge medications included pantoprazole and ondansetron for 3 days, along with symptomatic medications for nausea, acidity, constipation, diarrhea, pain/fever, and mucositis as needed.

C3 Day 1 on 06/04/2026 as per the protocol

Tab APRECAP 1 tab PO stat
Inj Dexa 8mg + Inj Pan 40mg + NS 100mlIv over 30mins
Inj Avil 2ml + Inj Hydrocort 100mg + NS 100ml IV over 30mins
Inj Paclitaxel (Mitotax) 100 mg + 5% Dextrose 250ml IV over 90 mins
Inj Carboplatin 180mg + 5% Dextrose 500ml IV over 2 Hours
100 ml NS Flush

07/04/2026 (Morning):
Patient initially planned for discharge. Advised electrolytes prior to discharge and continuation of anticoagulation (Clexane followed by Apixaban).
Patient developed acute desaturation (SpO_2 84–85% on room air) after barium swallow study, with mild respiratory distress and bilateral basal crackles. Oxygen supplementation initiated and discharge withheld.
Managed with oxygen (2 L/min), diuretics (Lasix), anticoagulation (Clexane), and antibiotics (Zavicefta + Aztreonam). Speech and swallow evaluation advised only pleasure pureed diet with strict aspiration precautions.

08/04/2026
Patient clinically improved, maintaining SpO_2 93–94% on 1.5 L/min oxygen. Steroids administered (Hydrocortisone). Hepatology review noted improving LFTs and INR (~1.3).
Patient stable for discharge. Oxygen requirement minimal (1–2 L/min). Antibiotics rationalized (Augmentin short course; Zavicefta stopped). Nebulization (Foracort, Duolin) and short steroid course advised. Follow-up planned with chest X-ray and labs.

Significant Events

MET call was raised

in view of Desaturation 88% on 5 litre
Oxygen requirement has increased to 7litre -SPO2-92%
Inj Hydrocort and Duolin nebulisation given as per ICU team
Oxygen level improved and now maintaining SPO2-92-93 % on 6litre

Condition at the Time of Discharge

Hemodynamically stable

Advice

Discharge Advice

Discharge advise from Dr. Darshit Shah

Discharge Medications-
Tab Pan 40mg PO 1-0-1 (30min before food)
Tab Emeset 8mg PO 1-1-1

SOS list ofmedications
Tab Emeset 8mg PO 1—1—1 SOS for nausea
Tab Flatuna 1tab PO 1—1—1 SOS for flatulence
Syp. Mucaine gel 10ml PO1----1----1 SOS for acidity
Syp. Cremaffin Plus30ml PO 0—0---1 SOS for constipation
Tab. Dolo 650mg 1tab PO SOS for fever/pain
Tab. Imodium 2mg 1tab PO SOS for diarrhoea
Mucopain ointment Local application 1—1 –1 SOS for mucositis

Discharge Advises from Dr. Talha Meeran
Inj Clexane 0.6ml 1-0-1 for another 2 weeks
After 2 weeks:
- Stop clexane
- Tab Apixaban 5mg 1-0-1

Tab Cordarone 200 1-0-1
Tab Aldactone 50 0-1-0
Tab Mexohar 150 1-1-1
Tab Tadalafil 20mg 0-0-1

Discharge Advises from Dr. Sushil Bindroo
Foracort neb 1mg 8am- 8pm followed by gargle
Duolin neb 10am- 10pm for 1 week
Inj effcorlin 50mg iv 8 hrly (1-1-1)for 3 days then stop
Inj augmentin 1.2g 12hrly(1-0-1)for 3 days then stop

OPD f/u on monday (13/04/2026)with chest Xray

Discharge Advises from Dr. Lajja Shah/ Dr. Misbah
Continue only pleasure pureed orally
Follow safe swallow strictly- counselled family about the same.
To continue exercises daily

Discharge Medication

Dosage Form & Drugs	Dose	Dosage unit	Admin route	Cycle	Duration	Remark
Tab Pan 40mg PO	40	mg	po	1-0-1	5 days	(30min before food)
Tab Emeset 8mg PO	8	mg	po	1-1-1	2 days	
Inj Clexane 0.6ml 1-0-1 for another 2 weeks	0.6	ml	SC	1-0-1	14 Days	Then start with -Tab Apixaban 5mg 1-0-1
Tab Cordarone	200	mg	PO	1-0-1	to conitnue	
Tab Aldactone	50	mg	PO	0-1-0	to conitnue	
Tab Mexohar	150	mg	PO	1-1-1	to conitnue	
Tab Tadalafil	20	mg	PO	0-0-1	to conitnue	
Foracort neb 8am- 8pm followed by gargle	1	mg	inhalational	1-0-1	7 days	
Duolin neb 10am- 10pm for 1 week	2.5	ml	inhalational	1-0-1	7 days	
Inj effcorlin	50	mg	IV	1-1-1	3 days	
Inj augmentin	1.2	gm	IV	1-0-1	3 days	

Patient Name	Saroj Agarwal	Patient	10663186

Follow Up

To follow up with Dr. Darshit shah for next Chemotherapy #C3 D8 on 13/4/2026
To follow up with Dr. Sushil Bidroo in OPD on monday (13/04/2026) with chest Xray

Urgent Care Instructions

Fever, Dysphagia, Pain and Breathlessness

- **In case of above symptoms or fever, pain or any uneasiness, please contact the the Emergency department at +91-22-6130-5005**
- **For Home Sample Collection please call us on 9324225522 from 8am to 8pm .**
- **"I hereby acknowledge that I have been explained the discharge summary in the language understood by me." .**

- ताप, वेदना किंवा कोणतीही अस्वस्थता असल्यास आणीबाणी विभागाशी +91-22-6130-5005 या क्रमांकावर कृपया संपर्क साधावा.
- होम सॅम्पल कलेक्शनसाठी 9324225522 या क्रमांकावर सकाळी 8 ते रात्री 8 या वेळात कृपया संपर्क साधावा.
- याद्वारे मला हे मान्य आहे की मला समजणाऱ्या भाषेत डिस्चार्ज समरी मला समजावून सांगण्यात आली आहे.

- बुखार, दर्द या किसी भी बेचैनी के मामले में कृपया +91-22-6130-5005 पर आपातकालीन विभाग से संपर्क करें।
- होम सैंपल कलेक्शन के लिए कृपया हमें 9324225522 पर सुबह 8 बजे से रात 8 बजे तक कॉल करें।
- "मैं एतद्द्वारा स्वीकार करता हूं कि मुझे मेरी समझ में आने वाली भाषा में डिस्चार्ज का सारांश समझाया गया"

- જો તાવ, દર્દ કે કોઈ અસ્વસ્થતા હોય તો કૃપા કરી ઈમરજન્સી વિભાગનો +91-22-6130-5005 પર સંપર્ક કરો.
- ઘરથી નમૂના ભેગા કરવા માટે કૃપા કરી સવારે 8થી રાત્રે 8 વચ્ચે અમને કૉલ કરો 9324225522 પર.
- હું અત્રે પહોંચ આપું છું કે મને સમજાયતે ભાષામાં ડિસ્ચાર્જનો સાર સમજાવવામાં આવ્યો છે.

Dr. Darshit Shah
Medical Oncology
Med. Council No.2023/10/7965
MBBS, D.N.B(General Medicine),D.M(Medical Oncology)